

How the Criminalization of Pregnancy Robs Women of Reproductive Autonomy

BY MICHELE GOODWIN

In 2003, the South Carolina Supreme Court upheld the conviction of Regina McKnight, an African American woman who was convicted at the age of twenty-two for committing “homicide by child abuse.”¹ She became the first woman in the United States to be arrested, prosecuted, and convicted for experiencing a stillbirth.²

Rather than an outlier case in the annals of American jurisprudence that stretched law beyond reason while restraining compassion and justice, McKnight’s conviction inspired similar prosecutions of other poor black women and then of other women. For example, in Mississippi, prosecutors charged Rennie Gibbs with “depraved heart murder” when her pregnancy—one month after her sixteenth birthday—resulted in a stillbirth.³ By state statute, Gibbs’s pregnancy was by default the product of statutory rape, given her age. Prosecutors ignored that, instead charging her with a crime that has an automatic life sentence in Mississippi. Similarly, Mississippi prosecutors charged Nina Buckwalter with depraved heart murder in the death of her fetus.

South Carolina expressed similar disdain toward dozens of black women, prosecuting them for pregnancy-based offenses.⁴ Prosecutors in Alabama,⁵ Arkansas, Florida, Indiana,⁶ New Mexico,⁷ Utah,⁸ and other states have similarly and selectively prosecuted pregnant women for crimes associated with their pregnancies. That is, but for their pregnancies, the women would not have been prosecuted for doing something wrong. In Iowa, a pregnant mother of two was jailed for three days after

falling down steps.⁹ Prosecutors initially suspected the pregnant mom’s slip and fall to be a calculated attempt to end her pregnancy. In Indiana, an immigrant was arrested and charged with first-degree murder after attempting suicide while pregnant. More than one-third of states consider pregnant women’s illicit drug use a form of child abuse,¹⁰ resulting in unprecedented forms of criminal and civil punishment.¹¹ Laws previously understood to protect pregnant women from domestic violence during pregnancy, such as fetal protection laws, now serve as the vehicles for prosecuting pregnant women.

Prior scholarship¹² emphasizes that in this “era of maternal policing, in addition to inspiring (and sometimes requiring) medical officials to breach confidentiality in the treatment of pregnant women,”¹³ the political turn toward scrutinizing women’s conduct during pregnancy “reshapes police interaction with pregnant women accused of crimes, motivates selective prosecution,¹⁴ and obligates judges to make poor judgment calls.”¹⁵ The case of *Ferguson v. City of Charleston* concerns such a policy, which required doctors and nurses to report pregnant women’s drug-test results to police without the women’s knowledge.¹⁶ Currently, thirty-eight states have implemented feticide statutes, a particularly worrying species of fetal protection laws.¹⁷

What these arrests, prosecutions, and laws expose are the multitudinous ways in which the criminalization of poor pregnant women—and normalization of that criminalization, such that even doctors and nurses are expected to participate in it—serve to humiliate women, interfere with their reproductive health, and ultimately rob women of reproductive autonomy. Criminalization reduces the range of options available to women. For example, as Carolyn Sufrin details in her new book, *Jailcare: Finding the Safety Net for Women behind Bars*,¹⁸ pregnant

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women's medical needs are profoundly neglected behind bars, and reproductive health care services for women caught in the criminal justice system are seriously limited. Moreover, securing public housing and employment after release can be almost impossible for women convicted of felonies. And criminalization further entrenches negative social and cultural norms like stigma, blame, and discrimination against women of certain social groups who may or may not be compromising their pregnancies. In addition, this expanding and escalating criminalization of pregnant women—whether framed as targeting negligent or intentional behavior or actions—includes attempted abortion, abortion, drug use, and accidents.

This essay addresses some of these concerns and highlights important related issues that deserve further attention. For example, according to a study issued by Lynn Paltrow, executive director of the National Association of Pregnant Women, and Jeanne Flavin, a Fordham University professor of sociology, hundreds of American women have been prosecuted for harming their pregnancies.¹⁹ The authors state that, in more than four hundred cases, “a woman's pregnancy was a necessary factor leading to attempted and actual deprivations of a woman's physical liberty.”²⁰ Their account of fetal protection interventions on pregnant women adds to the earliest literature in this field, examining the intersections of race and sex in policing women's reproduction.²¹ It is worth noting that the Paltrow and Flavin study accounted only for pregnancies between 1973 and 2005. Since that time, in Alabama alone, 479 women have been charged for violating that state's chemical endangerment law.²² The drug most often associated with their criminal charges was marijuana.

For many years, most scholars concerned about women's reproductive health have seemingly divorced the types of cases listed above from the canons of reproductive freedom, autonomy, and equality. Yet, these chilling cases, whereby women's reproductive autonomy suffers from criminal law interventions into reproductive health care, deserve scrutiny for what they portend politically, socially, legally, and medically. Reproductive autonomy must be understood to encapsulate not only the right to end a pregnancy but also the ability to experience a pregnancy without civil and criminal intervention, entrapment, and punishment by the state. To this end, scholarly approaches to reproductive autonomy should be mindful about and include a reproductive justice framework that takes seriously the full extent of women's and girls' capacities to control their own bodies.

This essay highlights Regina McKnight's prosecution as a cautionary tale for medical jurisprudence and for the possibility of reproductive autonomy for many U.S. women. It flags the increasingly dangerous zone that pregnancy occupies in American law and argues that criminalizing conduct

during pregnancy will ultimately undermine fetal health by inhibiting voluntary participation in prenatal medical visits. If women stand the risk of harassment and arrest while seeking medical care, the most vulnerable among them may well choose to go without, particularly as incarceration would also affect the lives of their children and families. Not only do women lose all semblance of reproductive autonomy as a result of pregnancy criminalization, but in jail or prison, women are forced to place their lives and, if they are pregnant, their fetuses' lives at risk.

In this essay, I first demonstrate how the criminalization of intended and unintended acts that prosecutors claim compromise pregnancy undermines or serves to limit reproductive autonomy and liberty. And while I anticipate that some scholars might argue that pregnant women would not be targeted by law enforcement agencies unless they actually acted illegally or in contravention of widely accepted social norms, such a claim ignores how poverty in pregnancy may trigger criminal suspicion. Second, I argue for the decriminalization of pregnancy. That is, I call for the abolition of pregnancy policing because this change will lead to better, more dignified outcomes for pregnant women, some of whom will become future mothers. My third primary aim in this essay is to explain that, ideally, abolishing pregnancy policing will relieve doctors and nurses of their problematic criminal detection and enforcement roles and return them to the practice of treating their patients and serving them with professionalism, empathy, trust, and compassion. Decriminalizing acts associated with pregnancy will ultimately relieve patients' fears that doctors prioritize law enforcement goals over providing prenatal services. Finally, decriminalization will also lead to better health and social outcomes for babies because their mothers will receive needed care and the babies will avoid the hardship of separation from their mothers and family, which in many states would involve foster care in infancy and possibly childhood.

How American Courts Began Criminalizing Pregnancy

In McKnight's case, prosecutors claimed that a South Carolina law that protects children under age eleven from depraved physical abuse or wanton neglect that results in death could be applied to McKnight's pregnancy. The law they referenced reads, “A person is guilty of homicide by child abuse” if he or she “causes the death of a child under the age of eleven while committing child abuse or neglect, and the death occurs under circumstances manifesting an extreme indifference to human life.”²³ This strategy was part of a new arsenal of approaches targeted at indigent pregnant women.

Vulnerable women in the United States cannot even begin to have meaningful reproductive autonomy while they are criminalized for pregnancies, reproductive choices, and birth outcomes that often may be beyond their control.

The case went largely unnoticed in popular media and public discourse, but it marked a watershed moment in U.S. law. The case reified and buttressed the criminalization of pregnancy. It established the troubling precedent that the outcome of a woman's pregnancy could be the subject of her criminal investigation, prosecution, and punishment for murder. For centuries, courts resisted this type of jurisprudence in both tort and criminal law because fetuses were not considered legal persons.²⁴ Courts spoke to the absurdity of establishing legal precedent whereby a fetus could sue her mother. More than a century ago, Justice Oliver Wendell Holmes, serving on the Massachusetts Supreme Court at the time, explained that any notion that a fetus "stands on the same footing as . . . an existing person" was absurd and illogical. For Holmes, there was no such thing as a "quasi independent life" of a fetus.²⁵ However, by the 1990s, drug war prosecutions involving crystallized cocaine (commonly referred to as "crack") reached out to target a new class of vulnerable suspects: pregnant women.

McKnight's prosecution also established conduct during pregnancy as a site for criminal law intervention, contributing to the normalization of police and prosecutorial involvement in women's reproductive health. In McKnight's case, the result was a prosecution for murder following a stillbirth; in subsequent South Carolina cases, law enforcement targeted poor women of color in the wake of healthy births, premature births, and miscarriages.

Third, McKnight's prosecution advanced a seriously distressing proposition related to perfection in pregnancy. Under this ruling, women's pregnancies could be held to a standard of faultlessness, whereby prosecutors can erroneously assume (as the court also did) that all pregnancies produce healthy babies and that absent so-called depraved conduct²⁶ on the part of pregnant women, stillbirths do not occur. The truth is that stillbirths occur for myriad reasons, including stress, hypertension, and high blood pressure.

Finally, McKnight's conviction served to politicize pregnancies, linking them with federal efforts to demonstrate toughness on drug crimes.²⁷ McKnight's conviction ensnared pregnancies in the so-called War on Drugs, with its lingering "tough on crime" and "tough on drugs" tropes. As one commentator wrote, "The inner-city crack epidemic is now giving birth to the newest horror: a bio-underclass, a

generation of physically damaged cocaine babies whose biological inferiority is stamped at birth."²⁸ McKnight's prosecution opened a new avenue for police and prosecutors to advance a disastrously unsuccessful campaign to reduce the incidence of illicit drug use by arresting women at prenatal visits, dragging them into police cars while shackled and handcuffed after delivery, and calling on doctors to disclose confidential medical records in the process.

The Undermining of Women's Health, Safety, and Security

McKnight was ultimately sentenced to twenty years in prison for her stillbirth. Prosecutors claimed that the twenty-two-year-old had a "depraved heart"—the legal standard for a homicide conviction—because she ingested an illicit drug during her pregnancy. McKnight had no prior convictions. She was a young, poor farm worker whose mother died the previous year as the result of a brutal hit-and-run accident. After she spent seven years in prison, her sentence was finally overturned in a unanimous decision.²⁹ In the end, the state never proved that the drug she ingested actually caused fetal demise.

However, by the time of her release, McKnight had suffered the shaming, stigma, and indignity of multiple trials, accusations that she was cold-hearted and indecent, and a lengthy confinement in prison. And because most states do not automatically expunge criminal records after acquittals and pardons, women like McKnight often suffer lingering consequences of incarceration, including difficulty in finding employment, housing, and even volunteering. These conditions after release further stigmatize and shame women. In recent years, researchers have documented such conditions as housing and health insecurity and vulnerability following incarceration, basically warning that homelessness is a stark consequence of punitive public policies.³⁰

How might the threat of criminal prosecution affect women's willingness to disclose to their doctors and nurses traumas associated with their lives and pregnancies? Indeed, might some pregnant women decide not to participate in prenatal care if they realize that law enforcement interests may supersede or even outweigh the goals of medical care and treatment? This is no idle concern. The United States

has both extremely high maternal mortality rates and stunning rates of mass incarceration. Coupled with the documented participation of medical personnel in pregnancy criminalization cases, the possibility of arrest and harm to a woman or a fetus throughout a pregnancy is quite serious. Women know this and may avoid care accordingly.

The United States ranks last among developed countries in maternal safety during pregnancy.³¹ That is, the United States lags behind France, Germany, Canada, the United Kingdom, Switzerland, Finland, Sweden, and a host of other industrialized, economically influential nations and also behind countries blighted by war, poverty, and recent genocides, like Serbia and Bosnia and Herzegovina. An American woman's likelihood of surviving her pregnancy is more like that of women in Saudi Arabia than women in the United Kingdom. In fact, Texas ranks last in the developed world for maternal mortality.³²

For all the aggressive criminal and civil state interventions in women's pregnancies in recent decades, key data from the World Health Organization report *Trends in Maternal Mortality: 1990 to 2008*³³ "place the United States fiftieth in the world for maternal mortality—with maternal mortality ratios higher than those of almost all European countries, as well as several countries in Asia and the Middle East."³⁴ Despite the fact that a majority of countries reduced their maternal mortality, a near double increase was reported for the United States.³⁵ In response to the WHO study, maternal health advocates issued a report declaring that the U.S. approach to maternal care is a costly human-rights failure.³⁶

Simply put, criminalizing pregnancy creates unhealthy norms in medical treatment that will chill prenatal screenings and ultimately harm pregnant women and their pregnancies. This toll will be particularly severe because of the fractured health care system and the severely constrained options for poor women of reproductive age.

Policies related to the criminal policing of pregnant women must also be viewed in broader public health contexts. The staggering rates of maternal mortality and the fact that black women are four times more likely to die than their white counterparts in pregnancy and childbirth raise the question, how safe is it to become pregnant in the United States? Compelling empirical data point out that pregnant women's lives are at risk in the United States.³⁷ There are multiple causes of high maternal mortality, including obesity, the later ages at which women become pregnant, the cesarean-section rate, and increasingly complex comorbidities in the population, some of which are related to obesity. These risks are compounded for some women by the escalation of targeted regulations of abortion providers (TRAP laws), decreasing access to medical providers, especially in rural communities (a result of some TRAP laws), and poor quality of care.

Nevertheless, high maternal mortality rates indicate the significant risks American women incur in bringing a pregnancy to term even with medical compliance. Comparatively, the U.S. rate of maternal mortality reveals a stunning policy failure. According to the Centers for Disease Control and Prevention (CDC), "[I]t is estimated that, between 1990 and 2008, 147 countries experienced a decline in maternal mortality ratio, 90 of which showed a decline of 40% or more. In 2 countries there was no change, and in the remaining 23 countries, including the United States, the maternal mortality ratio actually increased."³⁸ Given this, prioritizing state resources toward criminally tracking and prosecuting pregnant women above protecting and fostering public health is ill conceived.

Mississippi, where Rennie Gibbs was prosecuted for depraved heart murder, has one of the highest maternal mortality rates in the United States, and the rate is still rising.³⁹ The average rate of death for pregnant women in that state is about forty deaths per one hundred thousand live births. And in Texas, the maternal mortality rate nearly doubled between 2010 and 2014. Ironically, during that four-year period, Texas lawmakers enacted the most restrictive abortion laws in the country while also eliminating Planned Parenthood from its Medicaid-funded programs.

Conscripting Doctors and Nurses in Harming Pregnant Women

In the face of high rates of maternal morbidity and mortality, U.S. medical personnel should prioritize treatment and care of their pregnant patients. Sadly, medical personnel play crucial roles—intended or not—in the criminalization and punishment of pregnant women. In the overwhelming majority of cases involving the prosecution of pregnant women, information those women perceived as confidential was released to law enforcement officials by their medical providers without their permission and was then used against them in their trials. Medical visits triggered the arrests of Alicia Beltran (after she disclosed a prior reliance on pain medications)⁴⁰ and Christine Taylor (after her disclosure that she had considered abortion as an option early on in her pregnancy)⁴¹ and resulted in the threat of arrest of Lisa Epsteen (after she expressed interest in a vaginal birth over cesarean section).⁴²

In one striking example at the Medical University of South Carolina in 1988, medical staff members voluntarily released selected pregnant patients' confidential medical information to aid the prosecution of women suspected of drug use during pregnancy.⁴³ According to court records, MUSC officials profiled their patients, singling out pregnant black women to test for illicit drug use. Such actions rely on and perpetuate stereotypes and cultural biases to fulfill law enforcement rather than medical objectives.⁴⁴

The staff's actions were chronicled in the case of *Ferguson v. City of Charleston*,⁴⁵ which offers a chilling window into how medical providers can subordinate patients' interests to those of punishment and law enforcement officials. In that case, nearly a dozen women sued MUSC and local government officials for conducting warrantless and nonconsensual searches initiated and performed by MUSC medical staff members. The women were right; MUSC medical officials volunteered as informants against their female patients, initiating contact with a local prosecutor, Charles Condon, upon learning that he campaigned to extend child-abuse laws to women who had used drugs during pregnancy.⁴⁶ With the aid of medical staff, Condon launched an interagency task force consisting of prosecutors, hospital staff, and police, and the task force created a "Search Policy." Court records and internal memoranda provide an in-depth record of meetings between law enforcement officials and MUSC medical staff, including training sessions on how to collect urine samples for use in criminal investigations and on the importance of protecting key evidence to be used against pregnant patients. The task force devised reporting mechanisms. Police and prosecutors even trained the doctors and nurses and provided written guidance on "listing criminal charges that could apply to women coming under the Search Policy."⁴⁷

Ultimately, nurses and doctors at MUSC, along with police and prosecutors, arrested thirty women. Twenty-nine were African American. Medical staff members sought special dispensation for at least one white woman who met the criteria for arrest but was not arrested. For the only white woman arrested, racial profiling may nevertheless have contributed; a nurse and member of the interagency taskforce made a point of noting the following information in that patient's chart: "patient live[s] with her boyfriend who is a Negro."⁴⁸ This notation did not serve any medically relevant purpose, but it did reveal that an extralegal consideration—race—was involved in the implementation and enforcement of South Carolina's fetal protection law. At trial, the nurse admitted that she believed interracial relationships violated "God's way" and said that she had "raised the option of sterilization for black women testing positive for cocaine, but not for white women."⁴⁹ The court in *Ferguson* concluded that MUSC's horrific program "disproportionately targeted indigent, African-American women for search and arrest."⁵⁰

Physician and nurse involvement with the criminalization of pregnant patients introduces a profound level of cruelty into medical care. Transcripts in the *Ferguson* case reveal degrading interactions between medical staff and the poor black women they claimed to serve. Some of the arrested women were "denied the opportunity to change out of their hospital gowns or to make a phone call to family members to make arrangements for the care of their chil-

dren," and police arrested mothers who were "still bleeding, weak and in pain from having just given birth."⁵¹ Some women were handcuffed and shackled, with chains circling their abdomen. Leg irons were used in other cases. For any woman who could not walk, "a blanket or sheet would be placed over the woman, and she would be wheeled out of the hospital to a waiting police car and transported to jail."⁵²

When medical personnel adopt law enforcement objectives over pregnant patients' medical interests, they do so at the expense of their integrity, medical commitments, and obligations—not to mention the health and well-being of their patients.

Magnitudes of Disparate Harm: Racism and Pregnancy Criminalization

McKnight's case symbolizes the tragic intersections of harmful federal and state policies related to reproductive health care, drugs, and crime. The case exposes serious flaws in pregnancy-related prosecutions, particularly because prosecutors perceive risks of fetal harm as resulting *only* from maternal behavior. This notion of pregnancy and gestation is misinformed at best because it ignores research and reputable peer-vetted scientific literature indicating that poverty—as an isolated indicator—has a far more detrimental impact on fetal and childhood health outcomes than does a mother's ingestion of crack during pregnancy. Other factors, such as obesity, smoking cigarettes, and even drinking lemon-based nonalcoholic drinks during pregnancy can affect whether a baby is born healthy or not.

McKnight's story also dramatizes how racism, sexism, and stereotypes of the poor detrimentally influence perceptions of "good reproduction," let alone good mothering. As of this writing, despite the prosecutions of hundreds of women for fetal health outcomes, no man has ever been prosecuted for contributing to an unhealthy birth. Yet recent studies indicate that a father's age can influence fetal and child health,⁵³ and according to one study, increased paternal age is linked to autism and other disabilities.⁵⁴

Pregnant women experience different levels of medical care, privacy, and reproductive autonomy based on their socioeconomic status. The treatment of women using reproductive technologies that can pose significant risks to the fetus starkly illustrates these disparities. Compare the pregnancy of Brianna Morrison, a white woman who gave birth to sextuplets conceived using fertility treatments, to the womb policing of poor women and women of color taking place in states across the country. Morrison's pregnancy serves as one example among hundreds of high-order multiple births in the United States.

One year before McKnight was finally released from prison, Morrison gave birth to six babies after using power-

ful fertility drugs, including Follistim.⁵⁵ These drugs stimulate women's ovaries and are frequently linked to risky multiple births. Often in the case of high-order multiple gestations, doctors encourage their patients to selectively reduce the number of embryos or fetuses in the uterus. Doctors did so in this case, believing that most of the remaining fetuses would have a greater chance of survival to viability.⁵⁶ Without the procedure, there was a significant risk that some if not all of the babies would die.⁵⁷

Morrison and her husband declined to follow the medical advice provided by her doctor. They informed the public that the pregnancy was a miracle and that their path was to pray and surrender the outcome to God. Commenting on his wife's pregnancy, Ryan Morrison explained to a reporter, "[F]or us there's no difference between a fetus that's undeveloped and a baby."⁵⁸ The Morrison sextuplets were in critical condition after the delivery, subject to a battery of medical interventions to save their lives, including tests, treatments, respirators, and feeding tubes.⁵⁹ Less than two months after their births, all but one of the babies died.⁶⁰

The reaction to this tragedy was dramatically different from what McKnight experienced. For Morrison, there were prayers, interviews, loving blogs devoted to every update about her children's health status, websites, many donations, and sympathy. For McKnight, prison awaited following the death of her one fetus.⁶¹

Years ago, I asked, "Are these two women so different?"⁶² Clearly, no one called the police when Brianna Morrison refused to follow her doctor's advice. And law enforcement did not show up to arrest either Morrison or her husband when their babies began to die, despite the couple's refusals to follow medical advice. Yet arguably, Morrison took a higher level of risk in her pregnancy. Assuming good medical practice occurred during her fertility treatment, including securing informed consent, she was informed in advance of the magnitude of risks she faced by refusing to selectively reduce, including the risk of fetal demise with high-order gestations. Nevertheless, despite the fact that both women took some level of risk with the drugs they used in pregnancy, only women who, like McKnight, are poor or of color, become the targets of scorn, derision, and punishment.

In a 2002 report from the CDC, South Carolina was reported as one of the few states where over 70 percent of the births to women over thirty-five who had used in vitro fertilization resulted in multiples.⁶³ The report emphasized that "[assisted reproductive technology]-related multiple births are an increasingly important public health problem nationally and in many states."⁶⁴ For the years covered in the study, the multiple-infant birth rate for all live-birth deliveries was 3 percent; in South Carolina, IVF pregnancies produced multiples 56 percent of the time.

In South Carolina, 12 percent of births to women using IVF were triplet and higher-order, which is seventy-five times the national average, across all births, of 0.16 percent. Rates of triplets and higher-order multiples following IVF have dropped since then, but rates of twins remain very high. Rates of all multiples following fertility treatment are far above the national average and are associated with serious risks. These numbers are troubling because multiple births often produce low-birth weight babies and increase risks for miscarriage, spontaneous abortions, cerebral palsy, hearing and sight impairment, severe cognitive delays, chronic lung disease, and higher rates of postbirth morbidity.⁶⁵ In fact, low-birth weight babies are forty times more likely to die during the first few months of life,⁶⁶ and in South Carolina, multiple-birth babies resulting from the use of assisted reproductive technology are eight times more likely to be born low birth weight.⁶⁷

Ultimately, the fact that some women, like McKnight, are prosecuted for what prosecutors describe as risky behaviors, while others, like Morrison, are not, may evince motives other than protecting fetal health and life. A woman's ability or inability to experience pregnancy in line with her values and goals frequently maps along race and class status. Women of color or of lower socioeconomic status cannot act autonomously because their actions are policed and criminalized in ways that simply do not affect wealthier white women. Failure to be attentive to this profound reality disserves all women.

The Spread of the U.S. Incarceration Problem to Women and Children

In the past fifteen years, the problems highlighted in McKnight's case have intensified. The United States has by far the highest rate of incarceration of any country in the world.⁶⁸ Yet, female incarceration remains invisible in the United States, veiled in the broader realities of mass incarceration, hidden and often obscured by reports that cast this phenomenon as a black male problem. Women like McKnight, Melissa Rowland (prosecuted for refusing a cesarean section), Bei Bei Shuai (charged with first degree murder for attempting a failed suicide while pregnant), and others remain an unseen population in the narratives about mass incarceration and policing; campaigns to end selective prosecutions, abuse behind bars, and coercive plea bargaining virtually never feature women. Even narratives about prison rape address the suffering of men nearly exclusively, although the United States incarcerates more women than China, India, and Russia combined and women are also raped in prison.

One reason for the dramatic uptick in incarceration has to do with drug-related arrests and sentencing. Much of the nation's current incarcerated population, including wom-

en, are drug offenders—many of them first-time offenders. Children of incarcerated parents suffer an enormous toll through separation, stigma, and shaming. Children of incarcerated mothers bear the costs of displacement and institutionalization among strangers in foster care. Over 150,000 of the half million children in foster care are the children of incarcerated mothers snared in America's drug war. Unfortunately, children tucked away in U.S. foster care systems are more likely to experience physical and sexual abuse, drop out of school, become teenage parents, and enter the juvenile justice system.⁶⁹

McKnight's criminal conviction highlights mutual problems in medicine and criminal justice. It exemplifies the deeply entrenched stigmas and stereotypes associated with race and class in the delivery of reproductive health care and criminal justice in the United States. McKnight was a casualty not only of America's failed but ongoing drug war but also of the medical system itself.

While scholars, medical providers, and policy-makers consider what reproductive autonomy means in myriad contexts, we must be aware of, and attentive to, the criminal justice system's pernicious intervention in reproductive health care. The implications are significant for women, particularly in the United States, where maternal mortality is the worst among all developed nations. (Perhaps not surprisingly, the United States also ranks last among developed nations in the representation of women in political office.) Simply put, vulnerable women in the United States cannot even begin to have any kind of meaningful reproductive autonomy while they are criminalized for pregnancies, reproductive choices, and birth outcomes that often may be beyond their control. Despite constitutional jurisprudence anchoring women's reproductive privacy and autonomy as fundamental rights in American law, reproductive autonomy will remain more illusory than real in the lives of vulnerable women so long as class and race persistently define the contours of reproductive health care and rights.

1. S. Bhargava, "Challenging Punishment and Privatization: A Response to the Conviction of Regina McKnight," *Harvard Civil Rights-Civil Law Review* 39 (2004): 513-42, at 513.

2. *State v. McKnight*, 576 S.E.2d 168, 171 (S.C. 2003), *cert. denied*, 540 U.S. 819 (2003). See also S. Bhargava, "Challenging Punishment and Privatization," 513, 517.

3. See Associated Press, "Court to Hear Case of Woman Accused in Stillbirth," *Jackson Free Press*, April 1, 2013 (discussing the prosecutions of Rennie Gibbs and Nina Buckwalter for the deaths of their stillborn children).

4. *Ferguson v. City of Charleston*, 532 U.S. 67 (2001).

5. *Ex parte Ankrom*, 2013 WL 135748, at *22 (Ala., Jan. 13, 2013) (J. Parker, concurring) (stating, "the decision of this Court today is in keeping with the widespread legal recognition that unborn children are persons with rights that should be protected by law").

6. Ed Pilkington, "Indiana Prosecuting Chinese Woman for Suicide Attempt that Killed Her Foetus," *The Guardian*, May 30, 2012; Associated Press, "Chinese Immigrant Pleads Guilty in Baby's

Death," *ABC News*, August 2, 2013, <http://abcnews.go.com/m/story?id=19858146&sid=81>.

7. National Advocates for Pregnant Women, "Victory in the New Mexico Supreme Court" [press release], May 11, 2007, http://www.advocatesforpregnantwomen.org/whats_new/victory_in_the_new_mexico_supreme_court_1.php. As quoted in the press release, the state's attorney admitted in court that the law used to prosecute Cynthia Martinez's drug addiction during her pregnancy "could potentially be applied to pregnant women who smoked."

8. A. Sage, "Mother Denies Murder Charges," *Los Angeles Times*, March 14, 2004.

9. K. Hayes, "Did Christine Taylor Take Abortion into Her Own Hands?," *CBS News*, March 2, 2010, http://www.cbsnews.com/8301-504083_162-6255683-504083.html.

10. The states include Arkansas, Colorado, Florida, Illinois, Indiana, Iowa, Louisiana, Minnesota, Nevada, Oklahoma, Rhode Island, South Carolina, South Dakota, Tennessee, Texas, Virginia, and Wisconsin. "Substance Use during Pregnancy," Guttmacher Institute, October 1, 2017, <https://www.guttmacher.org/state-policy/explore/substance-use-during-pregnancy>.

11. That states (seventeen in total) consider illicit drug use and dependency to be a form of child abuse is unprecedented even though drug dependency is not a particularly new phenomenon. Stephen Randall discusses the high rates of heroin and opiate use in general among women in the late nineteenth and early twentieth centuries and legislative efforts to curb narcotics sales (S. R. Kandall, "Women and Addiction in the United States—1850 to 1920," in *Drug Addiction Research and the Health of Women*, ed. C. L. Washington and A. B. Roman (Rockville, MD: National Institute on Drug Abuse, 1998), 33-52, at 33, 34-35, 40, 45-46).

12. See, for example, M. Goodwin, "Fetal Protection Laws: Moral Panic and The New Constitutional Battlefront," *California Law Review* 102, no. 4 (2014): 781-75.

13. M. Angell and M. Greene argue that legislative restrictions impinge on fundamental reproductive freedoms "mainly by intruding on the relationship between doctor and patient" ("Where Are the Doctors?," *USA Today*, May 15, 2012, <http://www.usatoday.com/news/opinion/forum/story/2012-05-15/women-contraception-abortion-reproductive-rights-doctors/54979766/1>).

14. Linda Fentiman asserts, "The criminal prosecution of pregnant women for causing fetal harm exemplifies the . . . dangers of the American system of autonomous state prosecutors. Locally elected, politically ambitious, and largely unsupervised, individual prosecutors have wide discretion in deciding whether, when, and whom to prosecute" (L. C. Fentiman, "In the Name of Fetal Protection: Why American Prosecutors Pursue Pregnant Drug Users [and Other Countries Don't]," *Columbia Journal of Gender & Law* 18, no. 2 [2009]: 647-69, at 669).

15. *In re A.C.*, 573 A.2d 611, 617 (D.C. 1987), *vacated*, 573 A.2d 1235 (D.C. 1990) (upholding a court-ordered cesarean section of a terminally ill pregnant woman and concluding that the trial court judge "did not err in subordinating A.C.'s right against bodily intrusion to the interests of the unborn child and the state"); *In re Unborn Child of Samantha Burton*, No. 2009 CA 1167, 2009 WL 8628562 (Fla. Cir. Ct. Mar. 27, 2009) (granting an order authorizing Tallahassee Memorial HealthCare to provide "medically" necessary "to preserve the life and health of Samantha Burton's unborn child," including involuntary bed rest).

16. 532 U.S. 67, 83-86 (2001) (holding that a state hospital's performance of a diagnostic test to obtain evidence of a patient's criminal conduct for law enforcement purposes is an unreasonable search if the patient has not consented to the procedure).

17. See National Conference of State Legislatures, "Fetal Homicide State Laws," March 4, 2015, <http://www.ncsl.org/issues-research/health/fetal-homicide-state-laws.aspx>.
18. C. Sufrin, *Jailcare: Finding the Safety Net for Women behind Bars* (Oakland, CA: University of California Press, 2017).
18. L. M. Paltrow and J. Flavin, "Arrests of and Forced Interventions on Pregnant Women in the United States, 1973-2005: Implications for Women's Legal Status and Public Health," *Journal of Health Politics, Policy and Law* 38, no. 2 (2013): 299-343, at 300.
19. *Ibid.*
20. D. E. Roberts, "Punishing Drug Addicts Who Have Babies: Women of Color, Equality, and the Right of Privacy," *Harvard Law Review* 104, no. 7 (1991): 1419-82.
21. N. Martin, "The State That Turns Pregnant Women into Felons," *Alternet*, September 23, 2015, <http://www.alternet.org/drugs/when-womb-crime-scene>.
22. S.C. Code Ann. § 16-3-85(A) (West 2017).
23. *Dietrich v. Northampton*, 138 Mass. 14 (1884). See also *Magnolia Coca Cola Bottling Co. v. Jordan*, 124 Tex. 347, 359-60 (1935); *Allaire v. St. Luke's Hosp.*, 184 Ill. 359, 368 (1900); *Newman v. City of Detroit*, 281 Mich. 60, 62-63 (1937); *Stanford v. St. Louis-San Francisco Ry. Co.*, 214 Ala. 611, 612 (1926); *Buel v. United Rys. Co.*, 248 Mo. 126, 132-33 (1913); *Lipps v. Milwaukee Elec. Ry. & Light Co.*, 164 Wis. 272, 276 (1916); and *Gorman v. Budlong*, 23 R.I. 169, 176-77 (1901).
24. *Dietrich v. Northampton*, 138 Mass. 14, 16. See *Stallman*, 531 N.E.2d at 357, and *Bonbrest v. Kotz*, 65 F. Supp. 138 (D. D.C. 1946).
25. *Dietrich v. Northampton*, 138 Mass. 14, 16.
26. Under some state laws, a person can be found guilty of murder or other forms of homicide when, under circumstances evincing a depraved or morally corrupt indifference to human life, he or she recklessly engages in conduct that creates a grave risk of death to another person and that conduct causes the death of that person or another person (N.Y. Penal Law § 125.25[2] [McKinney]).
27. This brief essay does not unpack prior work on the drug war's casualties or its lingering effects, which continue to justify black women's arrests and prosecutions. For a more detailed analysis and discussion, see M. Goodwin, "Invisible Women: Mass Incarceration's Forgotten Casualties," *Texas Law Review* 94 (2015): 353-86; Goodwin, "Fetal Protection Laws"; Paltrow and Flavin, "Arrests of and Forced Interventions on Pregnant Women in the United States, 1973-2005," 303; P. A. Ocen, "Punishing Pregnancy: Race, Incarceration and the Shackling of Pregnant Prisoners," *California Law Review* 100, no. 5 (2012): 1239-311; Roberts, "Punishing Drug Addicts Who Have Babies"; M. Winerip, "Revisiting the 'Crack Babies' Epidemic That Was Not," *New York Times*, May 20, 2013; and G. Kolata, "Bias Seen against Pregnant Addicts," *New York Times*, July 20, 1990.
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29. *McKnight v. State*, 661 S.E.2d 354 (S.C. 2008).
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31. "The World Factbook," Central Intelligence Agency, accessed October 25, 2017, <https://www.cia.gov/library/publications/the-world-factbook/rankorder/2223rank.html>.
32. M. Redden, "Texas Has Highest Maternal Mortality Rate in Developed World, Study Finds," *The Guardian*, August 20, 2016.
33. World Health Organization et al., *Trends in Maternal Mortality: 1990 to 2008* (World Health Organization, 2010), http://whqlibdoc.who.int/publications/2010/9789241500265_eng.pdf.
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35. *Ibid.*; World Health Organization et al., *Trends in Maternal Mortality: 1990 to 2008*.
36. Bingham, Strauss, and Coeytaux state, "For a country that spends more than any other country on health care and more on childbirth-related care than any other area of hospitalization—US\$86 billion a year—this is a shockingly poor return on investment" ("Maternal Mortality in the United States," 189).
37. Centers for Disease Control and Prevention, "Infant Mortality Rates, by Race and Hispanic Ethnicity of Mother—United States, 2000, 2005, and 2009," *Morbidity and Mortality Weekly Report* 62, no. 5 (2013): 90.
38. *Ibid.*
39. J. Graham, "Pregnancy-Related Maternal Mortality, Mississippi, 2011-2012," Mississippi State Department of Health, Office of Health Data and Research, accessed October 25, 2017, msdh.ms.gov/msdhsite/_static/resources/5631.pdf; D. Paquette, "Why Pregnant Women in Mississippi Keep Dying," *Wonkblog*, *Washington Post*, April 24, 2015, at <https://www.washingtonpost.com/news/wonk/>.
40. D. Silva, "Shackled and Pregnant: Wis. Case Challenges 'Fetal Protection' Law," *NBC News*, October 24, 2013, http://usnews.nbcnews.com/_news/2013/10/24/21117142-shackled-and-pregnant-wis-case-challenges-fetal-protection-law?lite.
41. See B. Nichols, "Burlington Woman Will Not Be Charged with Feticide," *Radio Iowa*, February 10, 2010, <http://www.radioiowa.com/2010/02/10/burlington-woman-will-not-be-charged-with-feticide/>.
42. L. Stein, "USF Obstetrician Threatens to Call Police If Patient Doesn't Report for C-Section," *Tampa Bay Times*, March 6, 2013, <http://www.tampabay.com/news/health/usf-obstetrician-threatens-to-call-police-if-patient-doesnt-report-for/2107387>.
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44. MUSC screened for cocaine, but not other drugs like methamphetamines and opioids. See *Ferguson v. City of Charleston*, 532 U.S. 67, 70-73 (2001), and Roberts, "Punishing Drug Addicts Who Have Babies," 1471.
45. *Ferguson v. City of Charleston*, 532 U.S. 67.
46. Brief for Petitioners at 2-3, *Ferguson v. City of Charleston*, 532 U.S. 67 (2001) (No. 99-936).
47. *Ibid.*
48. *Ibid.*
49. *Ibid.*
50. *Ibid.*
51. *Ibid.*
52. *Ibid.*
53. See, for example, A. J. Wyrobek et al., "Advancing Age Has Differential Effects on DNA Damage, Chromatin Integrity, Gene Mutations, and Aneuploidies in Sperm," *Proceedings of the National Academy of Sciences of the United States of America* 103, no. 25 (2006): 9601-6; K. Kleinhans et al., "Paternal Age and Spontaneous Abortion," *Obstetrics & Gynecology* 108, no. 2 (2006): 369-77.
54. A. Reichenberg et al., "Advancing Paternal Age and Autism," *Archives of General Psychiatry* 63, no. 9 (2006): 1026-32. See also "Older Dads Six Times More Likely to Have Autistic Children," *EveningStandard*, September 4, 2006, <https://www.standard.co.uk/news/older-dads-six-times-more-likely-to-have-autistic-children-7214249.html>, and R. M. Goodman, "Problems in Medical

Genetic Services as Viewed from Israel," *Public Health Report* 99 (1984): 460-64.

55. See "Third Minnesota Sextuplet Dies," *Fox News*, June 18, 2007, <http://www.foxnews.com/story/0,2933,282732,00.html>. Follistim is produced using genetic recombinant technology and administered subcutaneously; see "Follistim AQ Cartridge," Merck & Co., Inc., accessed October 25, 2017, <http://www.follistim.com/>.

56. P. J. Smith, "Couple Chooses Life and Sextuplets," *LifeSite*, June 12 2007, <http://www.lifesitenews.com/ldn/2007/jun/07061207.html>.

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61. McKnight's sentence was later reduced to twelve years. See "Petition Filed Today Seeking U.S. Supreme Court Review of Unprecedented South Carolina Decision Treating a Woman Who Suffered a Stillbirth as a Murderer," National Advocates for Pregnant Women, May 27, 2003, at <http://advocatesforpregnantwomen.org>.

62. M. Goodwin, "Prosecuting the Womb," *George Washington Law Review* 76, no. 6 (2008): 1411.

63. Multiple births increase the risk of low-birth weight babies (L. A. Schieve et al., "Use of Assisted Reproductive Technology—United States, 1996 and 1998, Centers for Disease Control and Prevention," *Morbidity and Mortality Weekly Report* 51, no. 5 [2002]: 97-101). In South Carolina, approximately ten percent of annual births are to low-birth weight babies (Office of Public Health Statistics and Information Systems Division of Biostatistics and Bureau of Maternal and Child Health, *South Carolina Mother and Child Health Data Book 2013* [Columbia, S.C.: South Carolina Department of Health and Environmental Control, 2013], I-7, https://www.scdhec.gov/Health/docs/BiostatisticsPubs/MCH_2013.pdf).

64. Schieve et al., "Use of Assisted Reproductive Technology," 97.

65. D. BenEzra, "In-Vitro Fertilisation and Retinoblastoma," *Lancet* 361 (2003): 273-74. See also F. Bruinsma et al., "Incidence of Cancer

in Children Born after In-Vitro Fertilization," *Human Reproduction* 15, no. 3 (2000): 604-07, at 604, and R. Wimalasundera and N. M. Fisk, "In Vitro Fertilisation and Risk of Multiple Pregnancy," *Lancet* 359 (2002): 232. Bo Stromberg and his colleagues found that children born after IVF are more likely to need habilitation services than controls (odds ratio 1.7, 95% CI 1.3-2.2). For singletons, the risk was 1.4 (1.0-2.1). The most common neurological diagnosis was cerebral palsy, for which children born after IVF had an increased risk of 3.7 (2.0-6.6), and IVF singletons of 2.8 (1.3-5.8). Suspected developmental delay was increased fourfold (1.9-8.3) in children born after IVF. Twins born after IVF did not differ from control twins with respect to risk of neurological sequelae. Low-birth weight and premature infants were more likely to need habilitation than full-term babies (B. Stromberg et al., "Neurological Sequelae in Children Born after In-Vitro Fertilisation: A Population-Based Study," *Lancet* 359 [2002]: 461-65). "Sextuplets Spotlight Difficult Choice for Parents," MSNBC, July 27, 2007, <http://www.msnbc.com/id/19453745/>.

66. "Statistics," MedCost Maternity Education Program, accessed October 25, 2017, <http://www.medcost.com/prenatal/statistics.htm>.

67. See "Statistics," MedCost Maternity Education Program, accessed October 25, 2017, <http://www.medcost.com/prenatal/statistics.htm>; see also "Low Birthweight among Multiple Deliveries: South Carolina, 1995- 2005," March of Dimes, accessed October 25, 2017, <https://www.marchofdimes.org/Peristats/ViewSubtopic.aspx?reg=45&top=4&stop=182&lev=1&slev=4&obj=1>.

68. R. Walmsey, *World Female Imprisonment List*, 2nd ed. (International Center for Prison Studies, 2013), 1, www.prisonstudies.org/sites/default/files/resources/downloads/wfil_2nd_edition.pdf.

69. According to a recently released report by Human Rights Watch, many of these mothers will struggle even to make bail. The report, similarly to one released by the American Civil Liberties Union, explains that "when single mothers cannot bail out of jail, children may get sent to foster homes." The children of these incarcerated mothers "may go hungry or miss school or be left in the hands of family members incapable of caring for them" (J. Raphling, "'Mama's Bail Out Day' Campaign Aims to Help Jailed Mothers," Human Rights Watch, May 9, 2017, <https://www.hrw.org/news/2017/05/09/mamas-bail-out-day-campaign-aims-help-jailed-mothers>).